

ENGLISH FOR WORK / SEPTEMBER 2026

Healthcare Administration English

Conversation lab

8 complete workplace dialogues, followed by eight additional role-play cases. Study the exchanges, then make the conversation your own.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Find the right language. Speak, write, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For hospital administrators, clinic managers, practice administrators, care coordinators, patient-experience leaders, revenue-cycle staff, operations managers, and healthcare-adjacent professionals.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Inside this conversation lab

Original fictional training conversations in professional English. These are realistic scripts, not transcripts of actual people. The professional roles are the speaker labels; all figures, incidents, and organizations are invented.

01 Referral queue

02 A denied claim

03 Bed capacity huddle

04 Accreditation evidence

05 A vendor data request

06 Service recovery

07 Care coordination handoff

08 Board dashboard definitions

Then try eight independent role-play cases, followed by feedback criteria and references.

Read it. Hear it. Make it yours.

1. Read the setting. Identify the roles, the immediate problem, and what each speaker needs.
2. Read the whole conversation aloud with a partner. In a group, give a third person the observer role. For three-speaker scripts, assign each professional a separate voice.
3. Notice how the speakers ask a specific question, qualify a claim, disagree, explain a technical point, or agree on a next step. Underline language you would actually use.
4. Cover the script. Recreate the exchange using the situation and professional roles. Keep the meaning, but change the wording.
5. Switch roles and introduce a complication: a missing record, a different priority, an unavailable colleague, or a changed assumption. End with a clear outcome or unresolved question.

Register and terminology

These colleagues use concise professional language. In an internal technical meeting, familiar abbreviations can be natural. With a new colleague or a client, expand unfamiliar terms and check understanding. Keep disagreement directed at the evidence or proposal, and match the degree of certainty to what is known.

Independent study

Speak each role aloud. Pause before the reply and supply your own response before revealing the next line. Record a second version using invented details, then compare its clarity and tone with the script.

Professional context

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

FULL DIALOGUE 01 / 6 SPEAKING TURNS

Referral queue

A clinic reviews referrals awaiting appointments.

Roles: Access coordinator / Clinic manager

Access coordinator: We have 18 referrals waiting for triage. Four arrived without the supporting records.

Clinic manager: Separate incomplete referrals from those awaiting a clinical decision. They need different follow-up.

Access coordinator: Can scheduling decide which patients need the earliest slots?

Clinic manager: Clinical urgency belongs with the authorized clinical team. We can identify missing information and available capacity.

Access coordinator: I'll contact the referring offices through the approved channel and update each referral's status.

Clinic manager: I'll ask the triage lead to review the pending queue and confirm the escalation route.

Notice the professional language

Discuss this wording: "Can scheduling decide which patients need the earliest slots?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 02 / 6 SPEAKING TURNS

A denied claim

Revenue-cycle staff investigate a claim denial.

Roles: Billing specialist / Coding supervisor

Billing specialist: This claim was denied for missing authorization. Should I change the diagnosis code and resubmit?

Coding supervisor: No. Coding must reflect the documented encounter. First check the denial reason and authorization record.

Billing specialist: The authorization exists, but its reference wasn't included in the submission.

Coding supervisor: Verify that it applies to the service and dates before correcting the claim.

Billing specialist: I'll retain the denial notice and follow the payer's correction process.

Coding supervisor: Record the submission history so we can distinguish a corrected claim from a duplicate.

Notice the professional language

Discuss this wording: "This claim was denied for missing authorization. Should I change the diagnosis code and resubmit?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 03 / 6 SPEAKING TURNS

Bed capacity huddle

An admissions surge is affecting patient flow.

Roles: Bed manager / Nurse staffing coordinator

Bed manager: The dashboard shows six empty beds. Can we accept six more admissions?

Nurse staffing coordinator: Only three are currently staffed and ready. Two need cleaning, and one has a maintenance restriction.

Bed manager: I'll report physical beds separately from staffed available beds.

Nurse staffing coordinator: Good. I'll confirm the staffing position with the charge nurses before we discuss additional capacity.

Bed manager: Environmental services will give me the turnover status. I'll update the flow huddle after that.

Nurse staffing coordinator: Flag clinical priorities to the care team; the dashboard alone shouldn't determine placement.

Notice the professional language

Discuss this wording: "The dashboard shows six empty beds. Can we accept six more admissions?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 04 / 6 SPEAKING TURNS

Accreditation evidence

A quality team prepares evidence for an accreditation review.

Roles: Quality coordinator / Department manager

Quality coordinator: The checklist says annual training is complete, but three attendance records are missing.

Department manager: The staff say they attended. Can we count that as evidence?

Quality coordinator: We need to verify completion through the approved records, not backdate signatures.

Department manager: I'll check the learning system and ask the training coordinator about alternative documentation.

Quality coordinator: I'll mark the evidence gap clearly and record who owns the follow-up.

Department manager: If completion can't be verified, we'll arrange the required training and document the actual dates.

Notice the professional language

Discuss this wording: "The staff say they attended. Can we count that as evidence?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 05 / 6 SPEAKING TURNS

A vendor data request

An operations vendor requests a patient-level spreadsheet.

Roles: Operations manager / Privacy officer

Operations manager: The vendor wants names and diagnoses to investigate scheduling delays. May I send the full export?

Privacy officer: Let's establish the purpose, permitted access, and minimum information needed before sharing anything.

Operations manager: Could a de-identified summary answer the operational question?

Privacy officer: Possibly. We should assess that and the vendor's current agreements through our privacy process.

Operations manager: I'll ask them to specify the fields and intended use, without attaching patient data.

Privacy officer: Send me that request, and we'll confirm the approved method and scope of any disclosure.

Notice the professional language

Discuss this wording: "The vendor wants names and diagnoses to investigate scheduling delays. May I send the full export?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 06 / 6 SPEAKING TURNS

Service recovery

Patient-experience staff discuss a complaint about repeated cancellations.

Roles: Patient-experience lead / Scheduling supervisor

Patient-experience lead: The patient says we've canceled twice without explaining what happens next.

Scheduling supervisor: The first cancellation was clinician availability. The second was an unresolved referral issue.

Patient-experience lead: We need a clear explanation that doesn't blame the patient or disclose unnecessary information.

Scheduling supervisor: I'll verify the referral status and identify who can resolve the scheduling barrier.

Patient-experience lead: I'll acknowledge the disruption and offer a definite time for an update.

Scheduling supervisor: Once the barrier is resolved, my team will confirm available appointments directly with the patient.

Notice the professional language

Discuss this wording: "The patient says we've canceled twice without explaining what happens next." What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 07 / 6 SPEAKING TURNS

Care coordination handoff

Two teams clarify responsibility for a discharge follow-up.

Roles: Care coordinator / Clinic administrator

Care coordinator: The discharge note requests follow-up, but I don't see an accepted appointment request.

Clinic administrator: Our referral team received it this morning. Receiving it doesn't mean the appointment is booked.

Care coordinator: Who can confirm the next step and contact the patient?

Clinic administrator: I'll assign the referral coordinator and ask them to acknowledge ownership in the shared record.

Care coordinator: I'll pass on the clinical team's stated timeframe through the approved channel.

Clinic administrator: We'll escalate any inability to meet it to that team and document the response.

Notice the professional language

Discuss this wording: "Who can confirm the next step and contact the patient?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 08 / 6 SPEAKING TURNS

Board dashboard definitions

Administrators reconcile two reported waiting-time figures.

Roles: Performance analyst / Operations director

Performance analyst: Your slide says a nine-day wait. The access report says fourteen days.

Operations director: Mine measures referral receipt to first offered appointment. What does yours measure?

Performance analyst: Receipt to the appointment the patient actually attends. Different measures answer different questions.

Operations director: Then show both, with the reporting period and exclusions clearly labeled.

Performance analyst: I'll add the denominators and distinguish incomplete referrals from ready-to-schedule cases.

Operations director: Good. The board needs to understand the operational bottleneck before considering additional capacity.

Notice the professional language

Discuss this wording: "Mine measures referral receipt to first offered appointment. What does yours measure?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

Eight more situations to make your own

The following cases are open role plays. They give you facts, contrasting roles, useful expressions, and a short model response. Use what you learned from the complete dialogues to create a longer exchange.

Preparation: two minutes. Conversation: three to five minutes. Feedback: one minute. Switch roles and repeat with the second-round challenge.

ROLE-PLAY CASE 01

Patient Access, Scheduling, and Referrals

SHARED CASE

A referral is received without the requested specialty or reason for the appointment. The patient is expecting a booking confirmation.

Role A | Responding professional

Clarify missing information before responding. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Useful expressions

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Cover this until after your first attempt

Could you confirm the specialty and referral details through the approved channel? I can explain the scheduling process to the patient, but I cannot confirm an appointment until the required information is available.

Second round

The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 02

Revenue Cycle, Coding, and Denials

SHARED CASE

A dashboard groups rejected submissions and denied claims together. A manager wants to understand where follow-up work is needed.

Role A | Responding professional

Explain a useful distinction in plain English. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Useful expressions

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Cover this until after your first attempt

These categories describe different points in the process. Let's separate submissions that did not enter processing from claims that received a denial, then identify the follow-up required for each group.

Second round

Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 03

Patient Flow, Capacity, and Staffing

SHARED CASE

A unit reports 24 occupied beds and two planned discharges. The discharge timing is unconfirmed, but another team assumes two beds are available now.

Role A | Responding professional

Distinguish completed work from pending work and explain its impact. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to brief someone who has only one minute. Ask what is complete, what is open, and which open item affects the next action.

Useful expressions

We have completed ...; ... remains open.

As of ..., the status is

The next update will cover

Cover this until after your first attempt

There are 24 occupied beds. Two discharges are planned, but their timing is not confirmed. I'll distinguish current availability from expected availability in the capacity update.

Second round

The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 04

Quality, Safety, and Accreditation

SHARED CASE

A near-miss report identifies a scheduling error caught before the appointment. The review has not established how the error occurred.

Role A | Responding professional

Separate observations, assumptions, and conclusions. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Useful expressions

The available evidence shows

This may indicate ..., but

We have not yet established whether

Cover this until after your first attempt

The error was identified before the appointment. The cause is still under review. I'll record the sequence of events and the checks that caught it without assigning blame or an unconfirmed cause.

Second round

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 05

HIPAA, Privacy, and Information Governance

SHARED CASE

A colleague requests a full patient list for a limited administrative task. The appropriate scope and access have not been confirmed.

Role A | Responding professional

Make a request with a clear action, purpose, and realistic timing. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Useful expressions

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

Cover this until after your first attempt

Could you specify the purpose and fields required? Let's confirm the permitted access and necessary information through our privacy process before sharing the list.

Second round

The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 06

Patient Experience and Service Recovery

SHARED CASE

A patient has called twice about a delayed response. The administrator can investigate the communication delay but cannot make a clinical decision.

Role A | Responding professional

Acknowledge a communication problem and give a corrected message. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are frustrated because an earlier message created an expectation. Explain that expectation. Ask your partner to distinguish the correction from anything still uncertain.

Useful expressions

My earlier message did not make ... clear.

I'm sorry for The correct information is

Let me check that the revised explanation addresses

Cover this until after your first attempt

I'm sorry you have had to call again. I can check the status of your request and confirm the next contact. For the clinical question, I'll connect you with the appropriate care team.

Second round

The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 07

Population Health and Care Coordination

SHARED CASE

A discharge coordination note lists transport as pending. The next coordinator needs to know who is following up and what has been confirmed.

Role A | Responding professional

Give a handoff that the receiving person can confirm. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Useful expressions

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

Cover this until after your first attempt

Transport remains unconfirmed. The request is recorded, and the assigned coordinator is following up. Please check the latest status with that person and record any change before telling the patient a departure time.

Second round

The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 08

Executive Dashboards and Board Updates

SHARED CASE

A board chart shows waiting time falling from 40 to 30 minutes, but the reporting period changed from monthly to weekly.

Role A | Responding professional

Explain a useful distinction in plain English. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Useful expressions

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Cover this until after your first attempt

The reported waiting time is lower, but the periods differ. I'll label the time windows and prepare a comparable view before we describe the change as a sustained improvement.

Second round

Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and writes a 70-110 word message. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

HHS: HIPAA business associates

<https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/business-associates/index.html>

AHRQ: SBAR communication tool

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/sbar.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Writing feedback begins with your draft. Vocabulary, grammar and review prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-06. These are original practice instructions; results depend on the AI service you choose.

Rehearse with an AI colleague

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Healthcare Administration English

Professional audience: hospital administrators, clinic managers, practice administrators, care coordinators, patient-experience leaders, revenue-cycle staff, operations managers, and healthcare-adjacent professionals

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Dialogue 01: Referral queue

Original fictional setting: A clinic reviews referrals awaiting appointments.

Professional roles: Access coordinator / Clinic manager

Published dialogue (reference script, not my own performance):

Access coordinator: We have 18 referrals waiting for triage. Four arrived without the supporting records.

Clinic manager: Separate incomplete referrals from those awaiting a clinical decision. They need different follow-up.

Access coordinator: Can scheduling decide which patients need the earliest slots?

Clinic manager: Clinical urgency belongs with the authorized clinical team. We can identify missing information and available capacity.

Access coordinator: I'll contact the referring offices through the approved channel and update each referral's status.

Clinic manager: I'll ask the triage lead to review the pending queue and confirm the escalation route.

Vocabulary in this exchange: referral: Order, recommendation, or routing process that directs a patient or case to another clinician, service, or specialist.

END REFERENCE

TASK: Interactive workplace role-play

1. Use the supplied case or dialogue as the starting situation. Give a two-sentence briefing and offer two relevant professional roles for me to choose from. For a supplied dialogue, use its actual roles. Ask which role I want, then stop and wait. Do not write my replies.
2. After I choose, identify who you will play and the immediate communication goal. Play the other professional; where a meeting requires a third person, label each of your speakers clearly. Start with one natural workplace turn and wait for my reply. Keep most turns to one to three sentences and ask no more than one question at a time.
3. Let the exchange develop across six to ten learner turns, or end earlier if I type "feedback". Respond to what I actually say. Include a plausible clarification, disagreement or tradeoff without silently changing the starting facts. Mark any added constraint as a fictional second-round variation. Do not resolve approvals, evidence or commitments that remain uncertain.
4. Stay in role during the exchange. If my meaning is unclear, ask for clarification naturally. Give a hint only if I ask or cannot proceed. Do not deliver a model conversation in advance.
5. At the debrief, quote two of my phrases: one successful choice and one worth improving. Check factual accuracy, language, register and whether the next step was clear. Give up to three focused suggestions. Ask me to retry the weakest turn; wait. Only then offer an alternative wording and a harder replay.

END PROMPT

Copy this prompt online or choose another lesson and support level

Create more scenario dialogues

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

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Vocabulary in this exchange: referral: Order, recommendation, or routing process that directs a patient or case to another clinician, service, or specialist.

END REFERENCE

TASK: Extend the conversation library

1. Propose six distinct, common scenarios in this field beyond the supplied case or script. For each, name the setting, two or more professional roles, the communication problem and one useful terminology focus. Include routine coordination, clarification, a complication, disagreement, handoff and follow-up where relevant. Choose scenarios that fit this profession, rather than forcing unsuitable situations into it.
2. Ask me to choose one scenario, or request all six in sequence. Stop and wait. Do not write all the scripts before I choose.
3. For the selected scenario, write an original fictional dialogue of 12-18 substantial speaking turns between two or three professionals. Name each role, establish an actual work problem, and let the exchange progress through questions, clarification, competing constraints and a credible next step or explicitly unresolved issue. Use the occupation's natural nomenclature and register. Avoid an interview between a teacher and a learner, generic small talk, and inserting a glossary definition into every reply. Explain specialist terms outside the dialogue instead.
4. After the script, explain five useful expressions in context, identify two grammar or register choices and ask three questions about the speakers' reasoning. Withhold the answers until I attempt them. Add one role-switch challenge.
5. Before presenting a script, check names, numbers, chronology, roles and terminology for consistency. Label invented facts and acknowledge uncertain specialist usage. If I requested all six, deliver one complete script at a time and wait for "next". Make each scenario materially different.

END PROMPT

Copy this prompt online or choose another lesson and support level